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Correct Answer

Next Q

Practice Exam - 1

Question 26 of 117



A 66-year-old woman with dilated cardiomyopathy, LVEF 32% and Stage C chronic HFrEF returns to the office. She reports dizziness when getting out of bed and one episode of near syncope while walking. There were no arrhythmias detected by her implanted defibrillator. Current medications include: Sacubitril/valsartan 49/51 mg bid, carvedilol 12.5 mg bid, spironolactone 50 mg, empagliflozin 10 mg and torsemide 40 mg bid. On exam, BP 98/68, pulse 80 bpm, O2 sat 99% on room air. The jugular veins were not distended, lungs clear, heart regular with normal S1S2, no edema. Labs: Na 132, K 3.6, BUN 42, Creat 2.1, HCO3 30, Hb 12.4, NT proBNP 448.

Which of the following is the most appropriate next step in her management?



A. Lower torsemide to 40 mg daily



B. Lower sacubitril/valsartan 24/26 mg bid

C. Switch carvedilol to metoprolol 50 mg daily

D. Lower spironolactone 25 mg daily

E. Discontinue empagliflozin 10 mg

Commentary

Testing Point: The Board-certified AHFTC specialist should recognize appropriate treatment of symptomatic hypotension due to overdiuresis in a heart failure patient

Correct Answer: A. Lower torsemide to 40 mg daily

Rationale: A decrease in diuretic dosing in patients without congestion is appropriate before down titrating GDMT. Level of evidence for converting carvedilol to metoprolol to increase BP is not robust.

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